HORIZON · CONDITION INTELLIGENCE

One condition in. Two briefings out. Nothing unverified.

FOR STRATEGY

Six-section brief

Standard of care, pipeline, key players, and what each means for capital, service lines and drug spend.

FOR PROVIDERS

One-page card

Condition, quick summary, what changed, current drug resources. Boxed warnings mandatory.

BEFORE EITHER SHIPS

Independent review

A reviewer opens every cited source and can block. A named clinician releases the card.

PIPELINE

Condition in → 5 gatherers → card or brief → **fact-check** → clinician releases

BUILT AND RUNNING

200-condition catalog. Measured at $1.50 of model spend per card.

Informational reference. Not clinical decision support; not for the care of an individual patient.

The gate caught its first card

Type 2 diabetes mellitus. Twenty-six claims checked, all seven cited FDA label PDFs opened rather than trusted.

What the card said	What the label actually says
Metformin, with a link to its FDA label	That link resolves to a combination product of two drugs. A clinician clicking it lands on the wrong label.
Boxed warning lists renal impairment, sepsis, alcohol	The label also lists age 65 and over, and imaging with contrast. The contrast hold is a routine decision the card would not have prompted.
A kidney indication marked unverified	It is approved, and documented in the very label the card cites. The hedge became its own misinformation.

Verdict: BLOCK Release is unavailable to the reviewer until these are corrected. Three defects, ten further fixes, on a card that read well.

Informational reference. Not clinical decision support; not for the care of an individual patient.

What a card costs

$0.75	191K	65	13.8
model spend	agent tokens	tool calls	minutes to build

Step	Model	Cost
Research, three gatherers in parallel	Sonnet	$0.25
Card assembly	Sonnet	$0.09
Independent fact-check	Opus	$0.42

The fact-check is a third of the cost and 57% of the time. It is what caught the wrong-drug label. Moving it to a cheaper model saves twenty-five cents a card. Do not.

Measured on one card, 12 October 2026. Adds roughly $0.75 of orchestration, so about $1.50 all in.

Compute is not the constraint

The whole 200-condition catalog, carded and checked:

MODEL SPEND

$300

A rounding error against a single service-line decision.

CLINICIAN REVIEW

17–33 hrs

Fifteen to twenty times the model spend, in the scarcest time you have.

The first card needed a correction pass, so budget **one and a half to two review passes per card**, not one. And a queue of 200 unreviewed cards creates pressure to rubber-stamp, which is the single failure this system exists to prevent.

Review hours from 5 to 10 minutes per card. Apply your own loaded reviewer rate.

Release in waves

84	86	30
Tier 1 · card these first	Tier 2 · common	Tier 3 · specialty

1. Run a first wave of ten to fifteen tier-1 cards, sized so a reviewer clears a wave in one sitting.
2. Measure the real block rate across two waves instead of planning against a single card.
3. Set the pace from that number, and only then decide whether the full catalog is worth carding.

DECISION NEEDED One named clinical reviewer with standing time to review. Everything else is built.

Catalog is editorial, weighted to service-line relevance, not a ranked extract from a claims dataset.